

Fitness for Duty and Impaired Practitioner

Policy Number	POL-SAF-076	Owner / Department	Medical Staff / Human Resources
Effective Date	2024-01-10	Version	2.0
Last Revised	2026-04-18	Next Review	2028-04-18
Approved By	Medical Executive Committee	Applies To	All employees, licensed independent practitioners, allied health professionals, students, and contracted staff

1. Purpose and Scope

This policy protects patients and staff by ensuring that all personnel are fit to perform their duties and by providing a non-punitive pathway to identify, remove from duty, and refer impaired practitioners for evaluation and treatment. It is grounded in State licensing-board requirements, the medical-staff bylaws, and the facility practitioner-health program.

“Impairment” means an inability to practice or work with reasonable skill and safety due to substance use, a physical or mental health condition, or fatigue.

2. Identification and Reasonable Suspicion

- Any person who observes behavior suggesting impairment has a duty to report it to a supervisor, the administrator on call, or Medical Staff Services.
- Reasonable suspicion** is based on specific, articulable observations — slurred speech, odor of alcohol, unsteady gait, erratic behavior, or deteriorating performance — documented contemporaneously.
- A practitioner may **self-report** a condition confidentially to the Practitioner Wellness Committee without automatic disciplinary action.

IMPORTANT — If you reasonably suspect a colleague is impaired and patients are at risk, act immediately: remove the person from patient care and notify the supervisor or administrator on call now. Patient safety takes priority over the individual’s schedule.

3. Removal, Evaluation, and For-Cause Testing

- Relieve the individual of duties and arrange safe transport; do not allow them to drive if impaired.
- Arrange for-cause / reasonable-suspicion fitness-for-duty evaluation and testing per HR and medical-staff procedures.
- Refer to the Practitioner Wellness Committee or Employee Assistance Program for assessment and a monitored return-to-work plan.
- Determine reporting obligations to the State licensing board, consistent with bylaws and applicable law.

4. Return to Work and Confidentiality

Return to practice follows a documented, monitored agreement with defined conditions. Health information is handled confidentially and separately from routine disciplinary records, consistent with the non-punitive intent of the practitioner-health program and applicable law.

5. Severity & Reporting

Classify and notify per **POL-RM-013**. Impairment that contributes to a patient death or serious harm is a **SEV-1**; removal of an impaired practitioner from active patient care, or a credible threat, is a **SEV-2**; a fitness concern addressed before any patient contact, or a Near Miss, is a **SEV-3 / Near Miss**.

Severity	Notify	Deadline
SEV-1 (catastrophic / sentinel)	Medical Staff Office, HR, Risk Management, administrator on call	Within 1 hour
SEV-2 (major)	Supervisor, Medical Staff Office, and HR	Within 4 hours
SEV-3 / Near Miss (minor)	Supervisor / HR via electronic event report	Within 24 hours

If impairment presents as threatening or violent behavior, also follow the workplace-violence policy **POL-EH-011**.

6. References

- State licensing-board requirements for reporting and managing impaired practitioners.
- Medical-staff bylaws and the facility practitioner-health / wellness program.
- POL-RM-013 — Incident Reporting and Event Severity Classification.
- POL-EH-011 — Workplace Violence Prevention.